



PROCEDURE

Home and Community Visits

Scope (Staff):	All staff
Scope (Area):	CAMHS-wide

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

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Aim

To assist CAMHS Staff to identify and reduce health and safety risks when conducting home or community visits.

Risk

- Preventable incidents or accidents resulting in injury or harm to staff, family or children.
- Harm to the therapeutic relationship or future disengagement.

Background

Home and community visits (visits) may be organised because:

- they are built into a service's model of care (for example, in Multisystemic Therapy, Eating Disorders Service, Pathways, Crisis Connect Intervention, Acute Care Response Team (ACRT), and the Aboriginal Mental Health Service); OR
- A face-to-face appointment is clinically indicated but the child and their family are unable to attend a clinic appointment. The provision of a visit in these circumstances is subject to service capacity.

Visits may be single or recurrent. The purpose of a visit may be to:

- Establish and maintain a therapeutic relationship;
- Facilitate further therapeutic interventions;
- Observe family interactions;
- Enhance assessment and formulation;
- Enable more effective liaison with other agencies; OR
- Promote engagement with risky and difficult to reach children and families.

Definitions

- **Community visit:** When a CAMHS staff member provides a mental health service in any setting that is outside the physical location of the CAMHS site and is not a child's home or usual place of residence.
- **Child:** an infant, child, adolescent or young person under the age of 18 years.
- **Family:** any parent, legal guardian or identified carer of the child. For a child of Aboriginal or Torres Strait Islander descent, this includes a person within their kinship group who is recognised as their carer under customary laws and traditions¹.
- **Home visit:** When a CAMHS staff member provides a mental health service at a child's home or usual place of residence.

- **Personal Duress Alarm (PDA) / Safe-T Cards:** a device that, when activated, indicates that a person is under threat and requires immediate assistance. PDAs are wireless devices that should be worn when conducting visits.
- **Team verbal code:** a phrase or word used by a team to indicate an emergency during a visit.
- **Acute Care Response Team (ACRT):** a community hub-based service that primarily aims to provide access to timely, evidence-based, recovery focused, person-centred and trauma informed care for children and adolescents (and their families/carers) aged up to 18 years experiencing acute mental health episode who are non-active CAMHS clients.

Key Points

1. Two clinicians must always attend the first visit in any one episode of care.
 - **Note:** In exceptional circumstances, and subject to risk and safety assessments as outlined in this document, CAMHS clinicians may attend a child's **school** to conduct a first visit (assessment) on their own if:
 - i. The visit cannot take place at the child's home or any other community location.
 - ii. The visit must be approved by the relevant Service Manager and Head of Service.
2. Staff must assess safety before all visits. This includes gathering as much information as possible in relation to both client and environmental factors, in order to prepare for the visit.
 - The ACRT would use a Home Visit Risk Assessment Tool ([Appendix 1](#))
3. Staff must discuss any identified safety concerns with their line manager. A plan for the visit must be developed and documented. This includes a decision as to whether or not two staff members must undertake any subsequent/later visits and/or a phone call prior to each visit is required.
4. Before leaving a CAMHS service, visiting staff must provide the Line Manager, Head of Service or Administration staff with details of the visit/s.
5. Staff must have up to date training around emergency procedures and feel confident to follow these procedures in the event of an emergency.

Guiding Principles

- When conducting visits, cultural and social diversity must be taken into account. This includes considerations around Aboriginality, gender, ethnicity, pregnancy or maternity, disability, religion, spirituality and/or belief, sexual diversity and gender identity, socioeconomic status, trauma history, medical condition and age.
- During a visit, staff must respect the autonomy, customs, and cultural preferences of those residing or working in the home or community location.

When entering a home, staff must take into consideration cultural and societal norms. For example:

- Before a home visit, staff must contact the family and enquire about specific cultural or personal considerations regarding appropriate dress.
- Staff must also discuss relevant occupational safety and health (OSH) requirements with the family (e.g. no smoking in the house during the clinician's visit, acceptable footwear during the home visit, location of pets during the visit).
- When conducting a visit at a child's school, staff must obtain prior agreement from the school for the visit to take place and must comply with any relevant school policies while on the school site.
- Building rapport, trust and respect with a child and their family enables a balance between delivering a culturally appropriate service and ensuring the safety of staff.
- Acting in a respectful and person-centred manner during all visits will reduce risk to staff.
- During the visit, the home or community location is part of the staff member's work environment and as such, duty of care principles apply.

Equipment

- Government vehicle with first aid and fire safety kits, navigation equipment in both electronic and printed form (where available), Bluetooth functionality, and a reliable torch. If the vehicle does not have built-in navigation, a phone mounting device must be installed to ensure that staff can use the map function safely. Observation monitoring bag is to be taken if part of the ACRT.
- Personal Duress Alarm (SafeT card device) and sign-in / sign-out sheet;
- Work mobile phones with "location service" switched on (if possible)⁵;
- CAMHS Identification card; and
- Refer to:
 - [Appendix 2a – Home and Community Visits Recording Form \(excluding ARCT\)](#)
 - [Appendix 2b – ACRT Home and Community Visits Recording Form](#)

Process

Refer to the CAMHS Home and Community Visiting Flowchart (see [Appendix 3](#)) and the CAMHS Health and Safety Checklist (see [Appendix 4](#)).

1. Preparation

- Prior to the first visit, a full safety assessment must be conducted and documented in the medical record. The ACRT must use the Home Visiting Rating as per the [ACRT After Hours Work Instruction](#).
 - This includes an assessment of referral information, information in the child's medical record and on webPSOLIS (where available), and information from liaison with external agencies involved in the care of the child.
- If any of the following are identified, a plan for the visit must be discussed with the line manager and documented in the medical record.
 - Family / domestic violence has been perpetrated, and the perpetrator is at home or is likely to return home².
 - There is a risk of injury to patients or staff².
 - The staff member feels they may be at risk during the visit ².
- The safety assessment must be reviewed and re-documented:
 - When there is a change to the usual location of the visit (e.g. the client moves to new house) or circumstances in the home (e.g. family member moves into the house);
 - When a new risk is identified; and
 - When clinically indicated.
- Before every visit, the clinician must phone the family to confirm the details of the visit (e.g. location, who will be in attendance etc.) and document the phone call in the medical record. The phone call may highlight the need for a review of the safety assessment. The phone call may also be opportunity to start developing a therapeutic relationship with the family.
- Where a child has been referred from another CAHS team, safety must still be assessed as above. Where available the safety assessment completed by other CAHS teams/clinicians must be viewed and endorsed by the Multidisciplinary Team (MDT), Head of Service and Service Manager or ACRT (if applicable)
- Where teams have developed their own system for assessing safety risk prior to a visit, this must be documented in a work instruction for the team. For example ACRT to refer to [ACRT After Hours Work Instruction](#).
- The plan for the visit must consider:
 - whether or not the visit should take place;
 - possible alternative locations for the visit;
 - whether two staff members will be required for subsequent visits;
 - which staff members are required at the visit (for example, whether an Aboriginal Mental Health Worker is required);

- requirement for a chaperone when administering depot medication (see [CAHS Chaperones for Intimate Procedures policy](#)); and
 - timeframes around safety calls (particularly if the visiting staff member has not returned to base or is not responding to calls).
- The plan must be documented in the child's medical record.
- Where a visiting staff member plans to conduct subsequent visits on their own, this must be discussed with their line manager and the outcome of the discussion recorded in the child's medical record.
- Visiting staff member/s must provide administration staff with information about the visit. This includes:
 - Name, address and telephone number of child and family;³
 - Expected time of visit; and³
 - Expected time of return or completion of each scheduled visit³;
- Clinical staff / Administration staff must record the above information using [Appendix 2a – Home and Community Visits Recording Sheet](#) (excluding ACRT) or [Appendix 2b – ACRT Home and Community Visits Recording Sheet](#).
 - This document must be stored within the service's approved secure records management system. A link must be made available electronically to all clinical and administrative staff.
 - The Service Manager must ensure that the use of this document is included in the site orientation process for all new and acting staff members.
- When planning to visit Aboriginal or Culturally and Linguistically Diverse (CaLD) families, staff must:
 - Gather initial information in order to engage with the child and family respectfully and ensure culturally secure systems of care (refer to [Useful Resources](#) for guidance);
 - Assess the family's language needs at the outset, as this can help direct methods of consultation. Any language services that may be required for the visit should be pre-arranged in accord with the [CAHS Language Services policy](#);
 - Confirm with the child and family which family members they would like to be present for the visit; and
 - Liaise with relevant staff such as CAMHS Aboriginal Mental Health Workers as appropriate (refer to the [CAMHS Aboriginal Mental Health Service guideline](#) for further information).
- At minimum, staff who are conducting visits must have access to a work mobile phone with internet access / GPS / map functionality, for safe transit between locations.

- Services may have individual work mobile phone or communal work mobile phones. This is dependent on how often visits are conducted by the service.
- All staff must be orientated on how to use the phone in an emergency situation (particularly if using a communal phone).
- All phones must have easy, 'short cut' access to both 000 and the phone number for their base service.
- Wherever possible, clinicians should not use personal mobile phones for visits.
- Volunteers must not attend visits.
- Students must never conduct visits on their own.
- Psychiatric Registrars can conduct subsequent visits on their own if their clinical supervisor has deemed them competent to do so.
- Where visits are conducted after business hours, local processes must be developed, including contacting the manager at the end of the visit or in an emergency⁴.
 - ACRT staff to refer to the [ACRT After Hours Work Instruction](#)
- Personal Duress Alarms (PDAs) must be taken to all visits. All staff must complete the CAMHS Personal Duress Alarm – Safe-T Card training package prior to using a PDA (refer to [Required Training](#) section).
 - Before leaving the service to attend a visit, staff must sign out the PDA and turn the device on (refer to [CAMHS Personal Duress Alarm Procedure](#)).
- Where a service does not have access to PDAs, extra precautions should be taken with regards to communication between the staff member and administrative staff before, during and after the visit.
- Staff must be made aware of manned police stations in their local area, and these police stations should be made aware of the service being provided by CAMHS⁵.

2. Arrival, assessment and entering the site

- Prior to entering a home or community location, staff must:
 - Record a **Yellow Alert** on their PDA as per the [CAMHS Personal Duress Alarm Procedure](#). This voice recording acts as a verbal GPS marker.
 - Use [Appendix 5 - CAMHS Arrival at Home/Community visit Checklist](#) to assess whether or not it is safe to enter the site⁵.
- If the home or community location is not considered safe, staff must return to their service, report their concerns to their line manager, and complete a hazard report form and PSOLIS alert.

- When choosing a location to park their vehicle, staff must opt for a location that allows for easy access to the vehicle and rapid departure from the site (e.g. if safe to do so, park on the road rather than the driveway). Keys must be kept accessible at all times.
- When entering the site:
 - Prior to entering staff must establish their credentials and the reason for being at the site⁵.
 - Staff must establish the identity of the person to whom they are speaking and verify that the child and family are comfortable with them entering the site⁵.
- When invited to enter, staff must consider the layout of the site, including access and exit points such as doors and windows. Where possible, staff must position themselves such that they can exit quickly if required (e.g. between the child and family and the door);
 - Staff must be aware of potential obstacles or obstructions that can impede immediate departure in an emergency (e.g. toys, furniture, stairways etc.)
 - Staff must scan for, and remain aware of, potentially dangerous objects (e.g. screwdriver, knife, pens, pencils, bottles, pets etc).

3. Service delivery

- When providing the mental health service, the visiting staff members must:
 - Be aware that the situation can escalate at any time.
 - Pay close attention to warning signs and changes in the behaviour of any person present at the visit, such as:
 - Change of body language;
 - Change in tone of voice;
 - Change in content of speech; or
 - Non-accidental intrusion into staff member's personal space.
 - Be careful of slip, trip and fall hazards.
 - Avoid turning their back to the client and other people in the room.
 - Consider the needs of the carer/s and any other family members present at the visit.
- Staff must activate a **Red Alert** using the PDA any time they feel that their safety is at risk.
- For further information related to safe service delivery, refer to [CAHS Prevention and Management of Workplace Aggression and Violence policy](#) and [CAMHS Sexual Safety guideline](#)

4. Returning to CAMHS service

- On return to the car, the visiting staff member must notify the administration / person overseeing and monitoring the visit staff by phone call, SMS message, or in person to advise that visit is complete:
 - The follow-up timeframe for contact by the visiting staff member is 15 minutes following the expected finish time of the appointment.
 - If the 15-minute timeframe has passed after the expected appointment completion time and the person conducting the visit has not made contact to inform of any changes, administration / person overseeing and monitoring the visit staff must act as per the Emergency Steps When Clinician Has Not Returned to Base or Is Not Responding to Calls (see Appendix 6).
 - MST staff should refer to the Home and Community Visiting Work Instruction for contact requirements. ACRT staff to refer to ACRT Home and Community Visits and After Hours Work Instruction
- On return to the service, the visiting staff member must:
 - report back to the Line Manager and Administration staff;
 - review the management plan if there has been an incident;
 - Turn the PDA off and return it for charge and storage in a secure location; and
 - In the event where multiple visits are undertaken staff must call in after each visit e.g. ACRT
 - Complete the PDA sign out/in sheet.
- Administration / clinical staff (between 8am to 4pm) or the Clinical Nurse Consultant (between 4pm to 830pm) must:
 - Record that the staff member called to say that the visit has finished and/or returned to the service; and
 - Record that the PDA that was taken on the visit has been returned.
- The Line Manager/Service Manager must:
 - Check that all staff have returned from visits; and
 - Act as per flowchart in Appendix 6 - Emergency Steps When Clinician Has Not Returned to Base or Is Not Responding to Calls (excluding ACRT staff) when a staff member has not called at the end of a visit or not returned to base.
 - ACRT to refer to the ACRT Escalation Flowchart (Appendix 7)
- Provide support to staff after clinical incidents as per Support Strategies for CAMHS staff, CAMHS Clinical Supervision Policy, CAHS Clinical Incident Management Policy and CAMHS Sentinel Event Management Toolkit.

5. Car considerations

- All staff attending visits must comply with the [CAHS Motor Vehicle Fleet Management Policy](#).
- It is recommended that staff use government cars for home or community visits. Refer to [CAMHS Transportation of Mental Health Consumers Procedure](#).
- Staff must not transport children and families in personal cars. Using personal cars may present an OSH/security risk.
- Staff must gain approval from their line manager if they wish to use their personal car for visits.
- In the event a client deteriorates, an ambulance should be called.

6. Emergency considerations

- At any stage during the visit the staff member can make a clinical judgement that the situation has become unsafe and decide to leave⁵. Once this decision has been made, the staff member should attempt to leave in a polite, calm and orderly manner to avoid escalating the situation.
- If the staff member makes a clinical judgement that the situation has become unsafe but is unable to leave the premises, the staff member can activate a **Red Alert** using their PDA (refer to CAMHS PDA manual for guidance). If the staff member does not have a PDA, they may consider using their mobile phone to contact emergency services and / or their service.
- Team verbal codes may be used to indicate an identified safety risk in the home/community environment during a phone call back to the service. Verbal codes must be identified, clearly documented and communicated to all team members prior to use.
 - Example: To notify the team of escalating safety concerns in the home or community location, clinicians can discreetly alert the team to immediately contact the Police by making the following code statement: *"I forgot to send that fax to Trudy, please can you fax it off for me" for ACRT staff use the statement "please look in the red diary"*
- Once a staff member has left an unsafe situation and returned to their service, they must submit CAHS OSH Incident/Hazard Form.
- For more information, refer to [CAHS Bushfire Safety Policy](#), [CAHS Emergency Management Policy](#), [CAHS Incident and Hazard Reporting Policy](#) and Local Emergency procedures manual.

7. Required training

Staff must complete the following training – as appropriate to their role – prior to conducting any visit. All of these training packages can be accessed via [MyLearning](#).

- Aboriginal cultural eLearning;

- Aiming for Zero Harm;
- Basic Life Support;
- Clinical Skills Module 4 – Risk assessment and safety planning;
- Mandatory reporting of child sexual abuse (nurses and doctors only);
- Risk Management of Hazardous Manual Tasks (Theory);
- Personal Duress Alarm – SafeTCard;
- Sexual Safety eLearning;
- WA Mental Health Act; and
- Prevention of Workplace Violence and Aggression.
- ACRT to attend face to face education for Therapeutic Crisis Intervention

Related CAHS internal policies, procedures and guidelines
CAHS Home and Community Visits Policy
CAHS Work Health and Safety Policy
CAHS Bushfire Safety Policy
CAHS Child Safety and Protection policy
CAHS Clinical Incident Management Policy
CAHS Language Services Policy
CAHS Incident and Hazard Reporting Policy
CAHS Motor Vehicle Fleet Management Policy
CAHS Prevention and Management of Workplace Aggression and Violence
CAMHS Aboriginal Mental Health Service guideline
CAMHS Clinical Assessment Policy
CAMHS Clinical Supervision Policy
CAMHS Risk Assessment and Management Policy
CAMHS Sentinel Event Management Toolkit
CAMHS Sexual Safety Guideline

CAMHS Support Strategies for CAMHS Staff
CAMHS Personal Duress Alarm Procedure

References and related external legislation, policies, and guidelines
1. Mental Health Act 2014 (WA) , s.271
2. Australian Standard – 4485.2 – 1997 Security for Health Care Facilities (external services) Section 6.3.2
3. Australian Standard – 4485.2 – 1997 Security for Health Care Facilities (external services) Section 6.3.3
4. Australian Standard – 4485.2 – 1997 Security for Health Care Facilities (external services) Section 6.3.5
5. Australian Standard – 4485.2 – 1997 Security for Health Care Facilities (external services) Section 6.5
6. Occupational Safety and Health Act 1984 (WA)

Useful resources (including related forms)
Telethon Institute for Child Health Research/Kulunga Research Network and University of WA-Working Together: Aboriginal and Torres Strait Islander Mental Health and Wellbeing Principles and Practice
CAMHS Clinical Assessment policy, see appendix 1
Cultural Competence eLearning (accessed through MyLearning)
CAMHS Learning & Development Page
CAHS Workplace Hazard, Incident and Injury Reporting Page

This document can be made available in alternative formats on request.

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Appendix 1: Home and Community Risk Rating Tool

Risk Rating	Client factors	Environmental factors
Green / Low risk	• Patient and family/carer known to CAMHS • Patient and family/carer have no history of violence or aggression • Patient and family/carer are not known to regularly use drugs or alcohol • Patient's mental state is stable • Patient is compliant with medication • Patient and family/carer are cooperative with staff • Patient and family/carer will abstain from smoking during home visit	• Animals at the property can be moved to another area • There is safe access to the property • The property is within mobile phone reception
Amber / Moderate Risk	• Patient and family/carer new to CAMHS • Patient and family/carer have no recent history of violence or aggression • Patient and family/carer are not known to regularly use drugs or alcohol • Patient has minor deterioration in mental state or fluctuating presentation • Patient may be non-compliant with medication • Patient and family/carer are cooperative with staff • Patient and family/carer will abstain from smoking during home visit	• Additional people are present or residing at the home but staff are aware. • Animals at the property can be moved to another area • There is safe access to the property • The property is outside of mobile phone reception
Red / High Risk	• Patient and family/carer unknown to CAMHS • Patient and family/carer have known history of violence or aggression	• Additional people are present or residing at the home who may pose a risk of harm to staff. • Uncontained animals at the property

	• Patient and family/carer are known to regularly use drugs or alcohol • Patient is acutely distressed or psychotic • Patient is non-compliant with medication • Patient and/or family/carer are non-cooperative with staff • There are other known threats to staff	• There is no safe access and/or hazards on the property • The property is outside of mobile phone reception
Black / Extreme Risk	• Client has recently assaulted staff or policy	• Additional people are present or residing at the home who have recently assaulted staff or police. • There is no safe access and/or hazards on the property • Weather events such as cyclones, flooding, storms, bushfires.

Appendix 2a - Home and Community Visits Recording form (excluding ACRT)

Refer to the [CAMHS Home and Community Visits procedure](#) for instructions on completing this form. – See ACRT and update accordingly

Staff must carry a PDA at all times when conducting home and community visits.	When speaking on the phone to administrative staff, clinicians can discreetly alert the administrative staff to immediately contact the Police by making a verbal code statement. e.g. <i>“I forgot to send that fax to Trudy, please can you fax it off for me?”</i>	Act as per flowchart in Appendix 5 when a staff member has not called, sent an SMS, or returned to base within <u>15 minutes</u> of the expected completion time.
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Name of Employee/s attending the visit	Date	Employee mobile phone number (include both if two clinicians attending)	Client name	Client phone number	Client address	Appointment start time	Expected appointment completion time	Clinician has returned to based OR phoned / text to advise that the visit has finished? (Y/N)

Appendix 2b – ACRT Home and Community Visits Recording form

Staff must carry a PDA at all times when conducting all home and community visits.	When speaking on the phone to administrative staff / CNC, clinicians can discreetly alert them to immediately contact the Police by making a verbal code statement. e.g. <i>“Can you cancel my appointment in the Red Diary”</i>	Act as per the escalation procedure outlined when a staff member has not called or returned to base within <u>15 minutes</u> of the expected completion time.
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Date: _____ Car Registration Number: _____

Staff Names & Mobile Numbers	Personal Duress Alarm (PDA) number (One for each clinician)	PDA Status Checked Yes / No

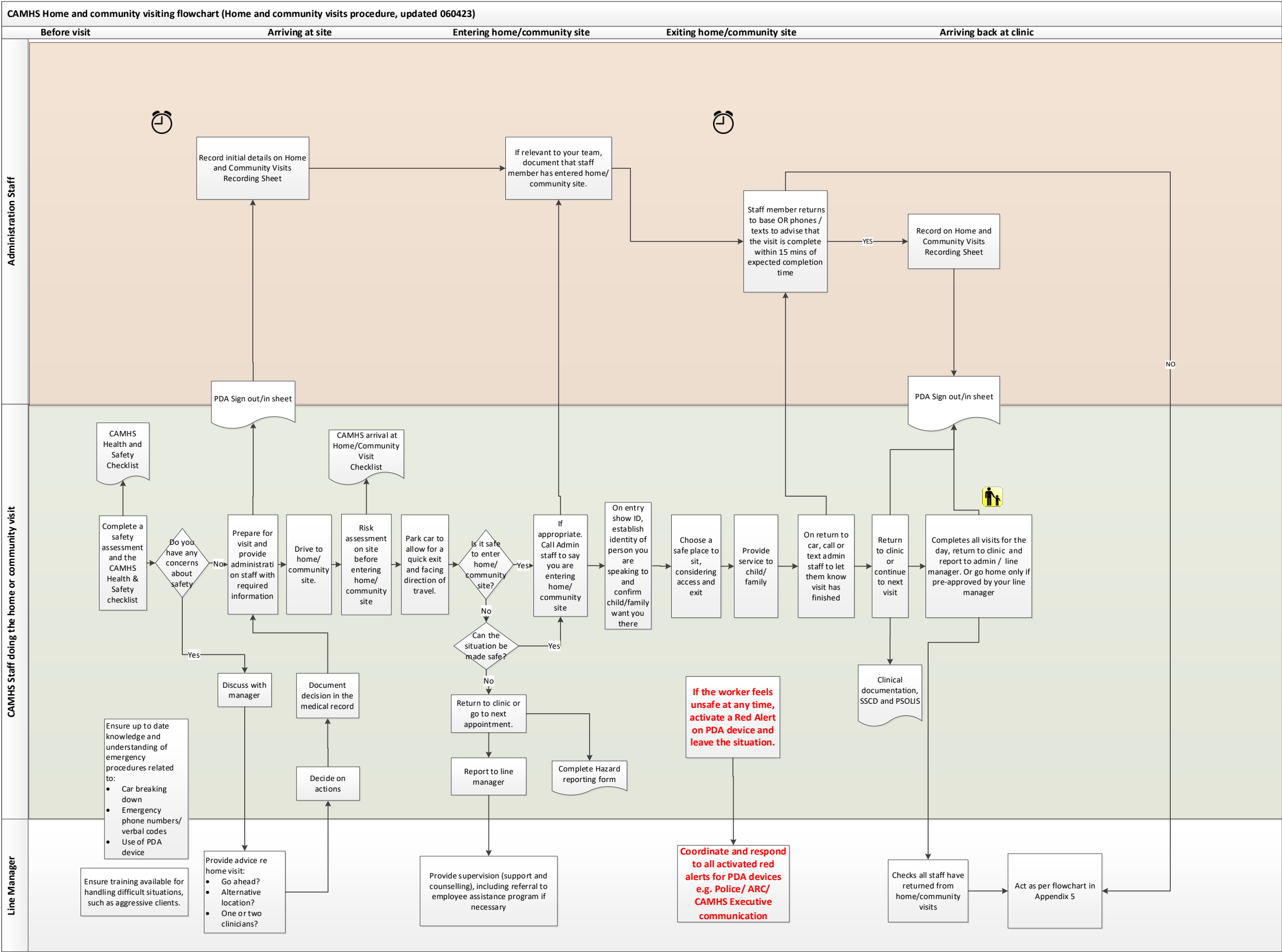
Client name & address (patient label)	Client phone number	Expected appt start time	Expected appt end time	Time of call-back to base	Escalation Y / N

I confirm that all staff are checked in:

Monitoring Staff Name: _____ Signature: _____ Time: _____

Clinical Nurse Consultant to retain copy of this document for audit purposes

Appendix 3 – CAMHS Home and Community Visiting Flowchart





Appendix 4: CAMHS Health and Safety Checklist

TASK		TICK COMPLETE
ADMINISTRATIVE CONSIDERATIONS		
Call family to confirm visit	Confirm address details of the visit	
	Confirm who will be present at the visit	
	Explain rationale of planned visit.	
	If possible, discuss their feelings about the visit.	
	Conduct brief safety checklist with the family.	
	Discuss any safety factors.	
	Complete COVID-screening / enquire about general health of the family (colds, flu etc.)	
	Explain the services preference on parking (i.e. on verge) and ask whether this would be an issue for the family or their neighbours.	
Communicate with administrative staff	Provide administrative staff with:	
	Name, address and telephone details of child and family	
	Expected time of visit.	
	Expected time of return or completion of visit. (See local procedures for visits occurring out of office hours or on the weekend)	
	Name/s of staff attending visit.	
	A contact number for workers during the visit.	
	Emergency contact number for workers (i.e. next of Kin).	
	Details of PDA device on Sign out/ in sheet.	

TASK		TICK COMPLETE
Make sure you take	Work phone (charged, turned on with pre-programmed emergency number)	
	Fully charged Personal Duress Alarm device.	
	CAMHS Identification card	
	If possible, staff member(s) doing the visit should minimise carrying any valuables	
The driver must	Check that the car has all required equipment:	
	First aid kit.	
	Fire safety kit.	
	Navigation equipment or map	
	Reliable / working torch	
	Familiarise themselves with the street on google maps.	
CLINICAL CONSIDERATIONS		
	Review the referral, child's medical record, PSOLIS and liaise with other external agencies where relevant to identify any environmental or physical factors that may impact on health and safety. Consider:	
	Use of drugs and/or alcohol at the premises	
	Use of weapons at the premises	
	Potentially dangerous animals at the premises (e.g. large dogs)	
	Unpredictable visitors at the premises	
	Likelihood of someone at the home becoming aggressive	
	Any other known safety risks (e.g. injury to staff or others)	
	Consider plans where visits are booked for after business hours	
	If your team uses a safety risk rating, apply this.	
	Discuss identified safety factors with Manager	

TASK		TICK COMPLETE
	Safety to be assessed and an action plan to be developed	

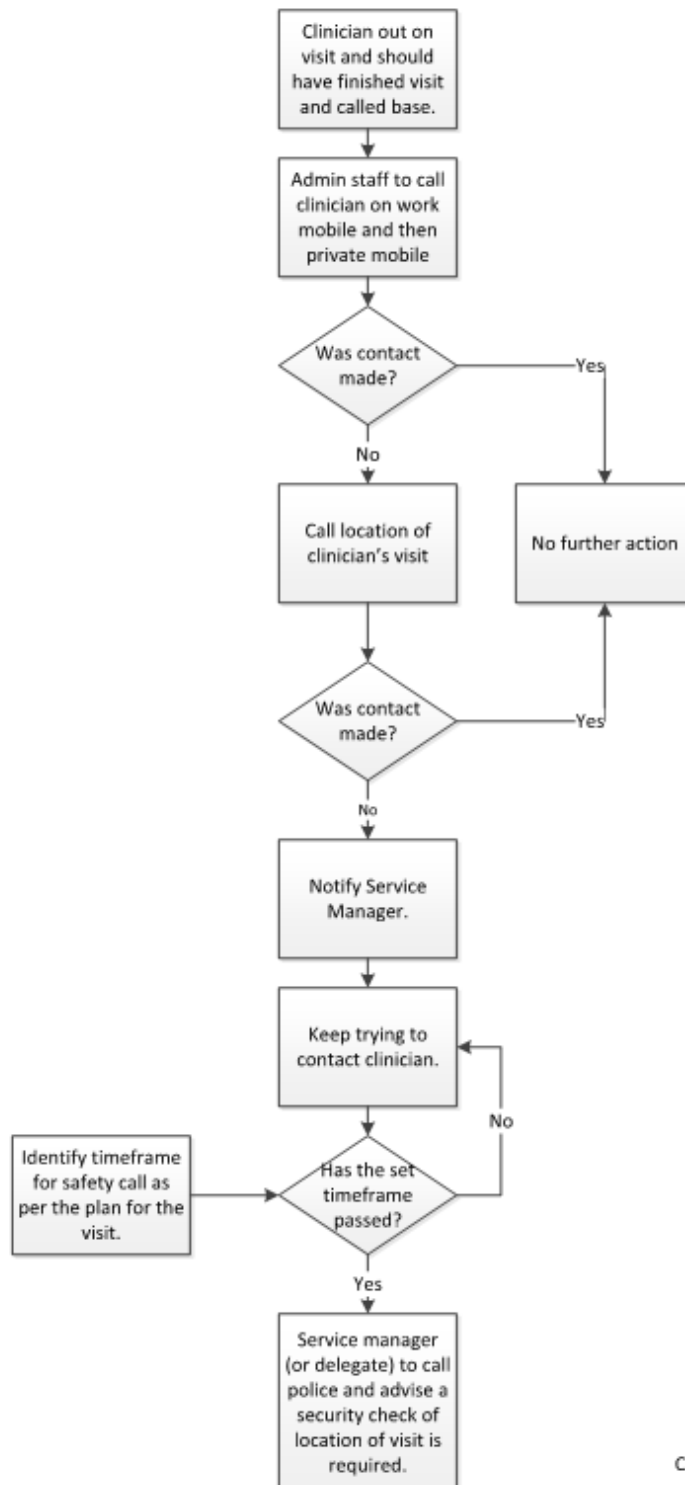
Appendix 5: CAMHS Arrival at Home/Community Visit Checklist

Please check these points when arriving to a home or community visit. You have the right to not enter the house or building or leave anytime you do not feel safe.

TASK		TICK COMPLETE
Parking the work car:	Park the car facing the best way for an emergency exit.	
	Park the car facing the way you are leaving so that the car does not have to be turned around.	
	Do not park in the driveway.	
	Lock car and keep keys on person.	
Valuables	Avoid carrying valuables into the home or building. Consider use of a lanyard with car keys and mobile phones attached or carry in a pocket.	
	Avoid leaving valuables visible in the car.	
Surroundings	Before leaving the car and/or walking up to the door, check/listen for obvious safety risks, such as:	
	Uncontrolled pets.	
	Heated arguments.	
	Unexpected visitors.	
Arriving at the site of visit	Record Yellow Alert on PDA device, and/or ensure smart phone location service is on to allow external tracking of staff location.	
	Keep phone, keys and PDA on person at all times.	
	If applicable, call the clinic administration staff making the safety calls before going into the house or building (check with service manager).	
When someone answers the door:	If it is the first time seeing a family:	
	Introduce yourself and any colleagues you are with	
	Show your identification	
	Say that you're with Child and Adolescent Mental Health Service.	

	Check who is home before entering, especially if door is answered by an unfamiliar person,	
	Wait to be invited into the home or building rather than just walking in.	
If you feel the situation is unsafe	Ask yourself if the situation can be made safe? (i.e. asking family member to put dogs on a lead)	
	If the situation cannot be made safe, you can make a clinical judgement that the situation is unsafe and decide to leave	
If you choose not to enter, or to leave the house or building	Return to the clinic	
	Report the unsafe conditions to your line manager verbally.	
	Complete a hazard report form to ensure follow-up action.	
	Complete a PSOLIS alert.	

Appendix 6: Emergency Steps When Clinician Has Not Returned to Base or Is Not Responding to Calls



CAMHS Home and community visits
procedure, endorsed 8.8.18

Appendix 7: ACRT Escalation Flowchart

Acute Care Response Team (ACRT) ESCALATION FLOWCHART

